STANDARD OPERATING PROCEDURE

Patient Medication Records Accuracy Documentation Compliance Review







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Department / Area: Pharmacy Operations — Patient Records & Compliance
Effective Date: September 2026
Version: 1.0
Review Cycle: Every 6 months, or upon regulatory update

1. Purpose

This procedure defines the standard process for creating, maintaining, and reviewing patient medication records to ensure accuracy, regulatory compliance, and audit readiness across daily pharmacy operations.

2. Scope

Applies to all patient-facing medication documentation, including prescription intake, dispensing records, counseling notes, and inventory-linked patient records. Relevant to pharmacy staff, pharmacists-in-charge, and any remote or administrative support handling patient documentation on the pharmacy's behalf.

3. Responsibilities

- Pharmacist / Documentation Lead: verifies accuracy of each record before it is filed or submitted.
- Support Staff / Virtual Assistant: enters, organizes, and flags records for review using the checklist in Section 6.
- Compliance Reviewer: conducts periodic audits per Section 6.

4. Definitions

- Audit-Ready Record: a record complete enough to withstand regulatory review without follow-up corrections.
- FEFO: First-Expired-First-Out, used to cross-check dispensing records against inventory dates.

- PHI: Protected Health Information, handled per HIPAA minimum-necessary and confidentiality standards.

5. Procedure

Step 1 — Intake

1. Confirm patient identity and cross-check against existing records to prevent duplicate or mismatched entries.
2. Record prescription details exactly as written: medication, dosage, prescriber, and date.
3. Flag any illegible, incomplete, or inconsistent information for pharmacist review before proceeding.

Step 2 — Documentation

1. Log the transaction in the records system within the same shift to maintain same-day accuracy.
2. Attach counseling notes where patient education was provided (e.g., dosage instructions, interactions discussed).
3. Store all PHI in accordance with HIPAA minimum-necessary access rules — no unnecessary duplication or exposure.

Step 3 — Verification

1. Cross-check dispensed medication against inventory using FEFO to confirm stock accuracy and catch discrepancies early.
2. Reconcile daily patient counts against logged records to confirm nothing was missed or double-entered.

Step 4 — Filing & Audit Readiness

1. File completed records in the designated compliance folder/system, labeled by date and patient ID.
2. Maintain a running log of any corrections made, with reason and timestamp, to preserve a clean audit trail.

6. Compliance & Quality Checklist

Use before closing out each shift or batch of records:

- Patient identity confirmed and matched to existing records
- Prescription details complete (medication, dose, prescriber, date)
- Counseling notes attached where applicable
- PHI handled per minimum-necessary access standard
- Inventory cross-check completed (FEFO)
- Record filed and labeled correctly

- Corrections log updated, if applicable

7. Escalation

Any discrepancy that cannot be resolved at the documentation stage (missing prescriber information, mismatched patient identity, inventory mismatch beyond normal variance) is escalated to the Pharmacist-in-Charge or Compliance Reviewer the same day, not batched for later review.

8. Revision History

- v1.0 — September 2026 — Initial version, created by Geneva P. Decano, RPh